

	A	B	C	D	E	F	G	H
1	Adrabetadex Provider Net-Cost Recovery Summary							
2	Client-ready economics readout for selected launch scenario							
3								
4	Product	Adrabetadex			Sourcing model	Buy-and-bill		
5	Scenario	Base Case: 340B COE HOPD, Buy-and-Bill			Pass-through status	Active		
6	Date generated	46135			Reimbursement 340B acquisition	WAC %		
7	Site type	HOPD				Primary 23.1%		
8	Payer type	Medicare						
9	340B status	Yes						
10	Coding status	Pass-through active						
11								
12	Per-dose economics							
13	Drug acquisition per dose	\$29,991.00						
14	Allowed drug reimbursement per dose	\$40,170.00						
15	Drug spread per dose	\$10,179.00						
16	Procedure revenue per dose	\$500.00						
17	Non-drug operating cost per dose	\$1,700.00						
18	Denial drag per dose	\$950.73						
19	Financing drag per dose	\$493.00						
20	Net operating margin per dose	\$7,535.27						
21	Break-even support per dose	\$0.00						
22	Annual economics							
23								
24	Annual administrations	26						
25	Annual drug acquisition	\$779,766						
26	Annual allowed drug reimbursement	\$1,044,420						
27	Annual non-drug operating cost	\$44,200						
28	Annual net operating margin	\$195,917						
29	Annual break-even support	\$0						
30	Cash exposure	\$179,946						
31	Annual 340B acquisition advantage	\$234,234						
32								
33	Margin, cash, and operational risk							
34	Margin risk	Low margin risk	Per-dose margin has stronger cushion under modeled assumptions.					
35	Cash exposure risk	Moderate cash exposure	Monitor outstanding administrations and remittance timing.					
36	Operational risk	Lower operational risk	Operational control posture is stable.					
37								
38	Where the value is							
39	340B acquisition lowers per-dose acquisition cost materially versus WAC.							
40	Pass-through timing improves launch-bridge payment visibility and shortens initial uncertainty.							
41	Clean PA, NDC, billing, wastage, and 340B documentation remain critical to realizing any margin.							
42								
43	Scenario interpretation							
44	Primary value is strongest in the 340B center-of-excellence / HOPD launch path; non-340B cases should remain clearly							
45	Permanent HCPCS timing is still important for long-horizon clarity, but it does not replace launch bridge controls.							
46	Keep operational requirements explicit: PA status, NDC reporting, invoice support, and claims edits quality.							
47								
48								
49								
50	Estimates are for planning purposes only and do not guarantee coverage, coding, payment, or reimbursement. Providers should validate payer-specific rules, payer edits, and operational controls before claim submission.							